

Agam - Ophthalmology Case Profoma



History:

Demographic details:

- Name:
- Age:
- Sex:
- Occupation:
- Locality:

Chief complains:

History of presenting illness:

The patient was apparently normal before _____, after which he/she developed

1. Decreased vision – Left or Right or Both eyes
 - a. Onset
 - b. Progression or non-progressive
 - c. Near or far vision
 - d. Diurnal variation
 - e. Aggravating and relieving factors
 - f. Associated factors
2. Mass on Right or Left eye
 - a. Onset
 - b. Progressive or non-progressive
 - c. Associated factors:
 - i. Diplopia
 - ii. Irritation
 - iii. Foreign body sensation
 - iv. Redness
3. Discharge
 - a. Onset
 - b. Duration
 - c. Amount of discharge

- d. Type of discharge
 - e. Diurnal variation
 - f. Aggravating and relieving factor
4. Other histories: (after describing chief complains)
- a. H/O Pain
 - b. H/O redness
 - c. H/O diplopia, polyopia
 - d. H/O photophobia
 - e. H/O colored halos
 - f. H/O flashes
 - g. H/O floaters
 - h. H/O glare
 - i. H/O trauma
 - j. H/O foreign body sensation

Past history:

- H/O similar complaints in the past
- H/O other ocular complaints
- H/O any systemic illness:
 - Diabetes mellitus
 - Hypertension
 - Asthma
 - Tb
 - Epilepsy
 - Coronary artery disease

Duration, treatment – regular or irregular and type of medication

- H/O surgical history

Personal History:

- Diet
- Sleep
- Appetite
- Bowel and bladder movements
- Alcohol, smoking

Family history:

- If any, to be mentioned

Examination:

General Examination:

- Built and nourishment
- Consciousness and orientation
- Vitals
 - Temperature
 - Blood pressure
 - Respiratory rate
 - Pulse rate
- Palor, icterus, clubbing, cyanosis, lymphadenopathy, pedal oedema

Systemic examination:

- Central Nervous System
 - Cranial nerves intact
 - Superficial and deep reflexes normal
- Cardiovascular System
 - S1 and S2 heard
 - No abnormal sound heard
- Respiratory System
 - Normal vesicular breath sound
 - No added breath sound
- Abdomen
 - No organomegaly
 - No tenderness

Ocular Examination:

- Head position
 - Tilt – [lagophthalmos](#), [ptosis](#)
 - Face turn - [squint](#)
 - Chin up/ down
- Facial symmetry

	Right Eye followed by left eye
Visual acuity	Improvement on pinhole?
Eyebrow	• Present or absent • Extent – med. To lat. Canthus In supra orbital ridge • Madarosis TB, Leprosy, radiation • Poliosis senile
Eyelids	Closed: ○ Eyelid proper

	• Lagophthalmos 7th nerve palsy • Check for bells phenomenon • Surface - scar, swelling (chalazion, hordeolum internum), ulcer, elevation, depression
	Open: • Interpalpebral fissure (9 to 11mm) • Ptosis(<9mm) • Lid Retraction (>11mm) ○ Lid margin • Ectropion, Entropion • Crusting, scaling, ulcer (ulcerative blepharitis) • Ant. Border – round • Post. Border – angulated • Meibomian gland duct opening – normal ○ Eye lashes • Madarosis, Poliosis • Trichiasis (trachoma), Districhiasis
Lacrimal Apparatus	• Punctum (lower) – visible • Signs of inflammation Dacryocystitis • ROPLAs test

Orbit and its contents

Orbit (margin of orbital inlet)	• Inspection – scar, irregularity, swelling • Palpation – irregularity, tenderness
Eyeball	Position, Visual access, Size, Movement
Conjunctiva	• Bulbar – color, congestion, chemosis, discharge, fleshy vascular mass, Bitot's spot • Palpebral – color, ulcer, swelling, concretions, papillae, follicle • Fornix – congestion, discharge,
Cornea	• Dimension • Edema • Sensation • Active ulcer – location, size • Scarring – location(peripheral/pupillary), size • Opacity
Sclera	• Color • Signs of inflammation • Traumatic perforation • Dilation/bulging
Anterior Chamber	• Depth • Eclipse sign

	• Hypopyon, hyphema • Abnormal constituents present or absent
Iris	• Present / absent • Colour • Pattern • Iris nodules iridocyclitis • Iridodonesis
pupil	• Number of pupil • Shape • Pupillary light reflex
Lens	• Present/ absent/ IOL • Transparency? • greyish white, milky white, pearly white, dirty white opacity

Provisional Diagnosis:

VIVA Questions

1. Occupation related diseases
 - a. Farmers – pterygium, cataract, traumatic fungal ulcers
2. Loss of vision
 - a. Sudden painful loss of vision
 - i. Acute angle closure glaucoma
 - ii. Iridocyclitis
 - iii. Trauma
 1. Chemical
 2. Mechanical
 - b. Sudden painless loss of vision
 - i. CRAO
 - ii. Massive vitreous haemorrhage
 - iii. Ischemic CRVO
 - c. Sudden painless defective vision
 - i. Optic neuritis
 - ii. Methyl alcohol amblyopia
 - iii. Central serous chorioretinopathy
 - iv. Non ischemic central vein occlusion
 - d. Gradual painful defective vision
 - i. Chronic iridocyclitis
 - ii. Corneal ulceration
 - e. Gradual painless defective vision
 - i. Progressive pterygium
 - ii. Corneal degeneration / dystrophy
 - iii. Developmental / senile cataract
 - iv. Optic atrophy
 - v. Chorioretinal degeneration
 - vi. Age related macular degeneration
 - vii. Diabetic retinopathy
 - viii. Refractive errors
3. Floaters
 - a. Vitreous hemorrhage
 - b. Vitreous degeneration
4. Photopsia – flashes of light
 - a. Posterior vitreous detachment
 - b. Vitreous traction band

5. Photophobia
 - a. Conjunctivitis
 - b. Keratitis
 - c. Anterior uveitis
6. Coloured halos
 - a. Intumescent cataract
 - b. glaucoma
7. Diplopia
 - a. Uniocular
 - i. Double pupil
 - ii. Keratoconus
 - iii. Incipient cataract
 - b. Binocular
 - i. Paralytic Squint
8. DD mass in the eye
 - a. Pterygium
 - b. Pseudo-ptyerygium
 - c. Pingicula
9. Discharge
 - a. Watery/ serous – viral
 - b. Mucoid – allergic
 - c. Stringy/ ropy – VKC/ spring catarrh
 - d. Mucopurulent – acute conjunctivitis, bacterial, chlamydial
 - e. Bloody (sanguinous) – gonococcal
 - f. Purulent – gonococcal
10. Colors of sclera
 - a. Yellow – icterus
 - b. Blue – cyanosis, thinning of sclera
 - c. Brown – melanin deposition
11. Corneal opacity grading
 - a. Nebula
 - b. Macula
 - c. Leucoma

d. Adherent leucoma